

● Pathology ● Sonography ● Digital X-Ray ● Colour Doppler ● ECG ● Echocardiography ● TMT ●

Patient ID :
Patient Name : MR.AYUSH JAISWAL 601068
Age/Gender : 30 Yrs / Male
Ref By : DR. SELF

Registered On : 13-Mar-2026
Sample Collected On :
Sample Reported On : 17-Mar-2026
Sample ID

Payment Status : **Paid**

HAEMATOLOGY

Test Name	Observed Values	Unit	Bio Ref. Interval
COMPLETE BLOOD COUNT (CBC - 5 Part)			
Hemoglobin	14.0	g/dl	13.5-17.5
R.B.C. Count	5.27	milli./cu.mm	4.5-5.9
Packed Cell Volume	46.1	%	37-53
Mean Cell Volume(MCV)	87.5	fl	80-100
Mean Cell Hemoglobin(MCH)	26.6	pg	26-34
Mean Cell Hb Conc(MCHC)	30.4	g/dl	32-36
RDW (CV)	13.3	%	11 - 16
RDW (SD)	41.8	fl	35 - 56
Total W.B.C Count	8080	/cumm	4500-11000
Differential Count			
Neutrophils	64	%	35-75
Lymphocytes	26	%	24-44
Monocytes	08	%	2-12
Eosinophils	02	%	0-6
Basophils	00	%	0-1
Absolute Differential Count :			
Absolute Neutrophils Count	5171	/ cumm	1575 - 8250
Absolute Lymphocytes Count	2101	/ cumm	1080-4840
Absolute Monocytes Count	646	/ cumm	90-1320
Absolute Eosinophils Count, AEC	162	/ cumm	0 - 660
Neutrophil-to-lymphocyte ratio, NLR	2.5	%	0.78 - 3.53
Platelet Count	2.68	Lakh/cumm	1.5-4.5
Mean Platelet Volume (MPV)	10.4	fl	6.5 - 12.0

Sample Type: EDTA Whole blood

Performed on:- "Mindray BC-6200" Fully Automated 5 Part Hematology Analyzer.

Methods: - Hemoglobin by colorimetric method. Total RBC count, platelets count by electrical Impedance method. PCV, MCV, MCH, MCHC: calculated, RDW-CV by RBC Histogram, WBC Parameters by Laser scatter fluorescence flow cytometry/microscopy, MPV, PDW by platelet Histogram IPF by PLTO scattergram. Reference: - Nathan DG, and Oski, FA(1981) Hematology of Infancy and Childhood, ed 2, WB Saunders, pp1552. Dallman PR (1977) pediatrics, 16th edition, Appleton-Century-Crofts, p 1111 pediatrics Normal Range (1995) Children's Hospital of Buffalo, Coulter Viewpoint, No.17. p 8 Pediatrics Normal Range Study (1995) Children's Hospital, Minneapolis and St. PAUL.



● Pathology ● Sonography ● Digital X-Ray ● Colour Doppler ● ECG ● Echocardiography ● TMT ●

Patient ID :
Patient Name : MR.AYUSH JAISWAL 601068
Age/Gender : 30 Yrs / Male
Ref By : DR. SELF

Registered On : 13-Mar-2026
Sample Collected On :
Sample Reported On : 14-Mar-2026
Sample ID

Payment Status : **Paid**

BIOCHEMISTRY

Test Name	Observed Values	Unit	Bio Ref. Interval
Blood Sugar Random	83	mg/dL	Normal : <140 Diabetic : ≥200 + Symptoms

Sample Type : Fluoride Plasma

Method : GOD-POD

Treatment Goals for Adults with Diabetes-

1)Pre-Prandial plasma glucose (FBS) 80-130 mg/dl

2)Post-Prandial plasma glucose (PPBS) <180 mg/dl

3)HbA1c <7.0%

4)Blood pressure <140/90 mmHg

*Source: Adapted American Diabetes Association: Diabetes Care 41(suppl 1):s86, 2018.

● Pathology ● Sonography ● Digital X-Ray ● Colour Doppler ● ECG ● Echocardiography ● TMT ●

Patient ID :
Patient Name : MR.AYUSH JAISWAL 601068
Age/Gender : 30 Yrs / Male
Ref By : DR. SELF

Registered On : 13-Mar-2026
Sample Collected On :
Sample Reported On : 17-Mar-2026
Sample ID

Payment Status : **Paid**

Urine Routine Examination

Test Name	Observed Values	Unit	Bio Ref. Interval
Physical Examination			
Quantity	30 ML		
Colour	Pale Yellow		Pale Yellow
Appearance	Clear		Clear
Chemical Examination			
Protein	Absent		Absent
Glucose	Absent		Absent
Ketones	Absent		Absent
Bile Salt / Bile Pigment	Negative		Absent
Occult Blood	Negative		Absent
Microscopic Examination			
Pus Cells	1-2	/ HPF	0-5
Epithelial cells	1-2	/ HPF	
Red Blood Cells	Nil	/ HPF	Nil
Spermatozoa	Absent		Absent
Casts	Absent		Absent
Crystals	Absent		Absent
Yeast Cell	Absent		Absent
Bacteria	Absent		Absent

Sample Type : Urine

Method: Urinalysis-Multiple Reagent Strip Method & Microscopy done after centrifugation of 2500 rpm for 5 minutes. Glucose oxidase-peroxidase, protein - Tetrabromophenol blue, Ketone-Sodium nitroprusside, Blood-Peroxidase activity of hemoglobin, Bilirubin-Diazotized dichloroaniline, Urobilinogen - Ehrlich Method.

Please look for drug history (SGLT-2 inhibitors) in diabetic patient if urine sugar is irrational high.

END OF REPORT

(# For test performed on specimens received or collected from non-SDC locations, it is presumed that the specimen belongs to the patient named or identified as labeled on the container/test request and such verification has been carried out at the point generation of the said specimen by the sender. SDC will be responsible Only for the analytical part of test carried out. All other responsibility will be of referring Laboratory.)


Dr.Vidhi Rath Maheshwari
MBBS,M.D
Lab Director, Pathologist